

Why does this happen? If fevers are beneficial, why do we fight them so universally?

I don't think it's complicated: Fevers hurt. And people don't want to hurt. That's it.

A doctor's goal is not just to cure disease. It's to cure disease within the confines of what's reasonable and tolerable to the patient. Fevers can have marginal benefits in fighting infection, but they hurt. And I go to the doctor to stop hurting. I don't care about double-blind studies when I'm shivering under a blanket. If you have a pill that can make a fever stop, give it to me now.

It may be rational to want a fever if you have an infection. But it's not reasonable.

That philosophy—aiming to be reasonable instead of rational—is one more people should consider when making decisions with their money.

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Academic finance is devoted to finding the mathematically optimal investment strategies. My own theory is that, in the real world, people do not want the mathematically optimal strategy. They want the strategy that maximizes for how well they sleep at night.

Harry Markowitz won the Nobel Prize for exploring the mathematical tradeoff between risk and return. He was once asked how he invested his own money, and described his portfolio allocation in the 1950s, when his models were first developed:

I visualized my grief if the stock market went way up and I wasn't in it—or if it went way down and I was completely in it. My intention was to minimize my future regret. So I split my contributions 50/50 between bonds and equities.

Markowitz eventually changed his investment strategy, diversifying the mix. But two things here are important.

One is that “minimizing future regret” is hard to rationalize on paper but easy to justify in real life. A rational investor makes decisions based on numeric facts. A reasonable investor makes them in a conference room surrounded by co-workers you want to think highly of you, with a spouse